

TABLE 4

Ineffective Treatments for Cold Symptoms in Adults

Treatment	Study type	Findings
Antibiotics	2 Cochrane reviews of 11 RCTs each ^{46,47}	No benefit for symptom duration or severity compared with no antibiotics or placebo
Antihistamine monotherapy (sedating and nonsedating)	2 Cochrane reviews of 18 RCTs and a subsequent RCT ^{6,34}	No more effective than placebo for global improvement
Antitussives and expectorants*	Cochrane review of 10 trials without meta-analysis ⁶	No more effective than placebo for cough
Codeine	Older RCT of 82 adults ⁴⁸	No more effective than placebo for cough
Echinacea	Cochrane review of 24 RCTs without meta-analysis ⁴⁹	No more effective than placebo for reducing symptom duration or severity
Intranasal corticosteroids	Cochrane review of 3 RCTs ⁵⁰	No more effective than placebo for reducing symptom duration or severity
<i>Pelargonium sidoides</i> (African geranium)	Cochrane review of a low-quality trial ⁵¹	No significant improvement by day 5
Steam	Cochrane review of 6 trials with 387 participants ⁵²	No benefit of using heated humidified air for treatment of URIs
Vitamin C	Cochrane review of 29 RCTs ¹⁶	No more effective than placebo for reducing symptom duration or severity once symptoms have developed
Vitamin D	RCT of 322 adults ¹⁹	No difference in URI occurrence, days of missed work, or symptom duration or severity with high-dose vitamin D supplementation (200,000 IU followed by 100,000 IU monthly)
Vitamin E	RCT of 652 adults 60 years and older ⁵³	Significantly worse URI symptoms with daily supplementation of vitamin E (200 mg) compared with no supplementation; no difference in URI occurrence

RCT = randomized controlled trial; URI = upper respiratory tract infection.

*—The American College of Chest Physicians does not recommend the use of antitussives or expectorants.⁵⁴

Information from references 6, 16, 19, 34, and 46-54.